



THE RETREATING STATE

- **Health is on the State List (List II) of the Constitution; the primary responsibility is with states**
- **Role of state is crucial, and health a key factor for economic and social development**
- **Public spending on health has risen to 1.8% of GDP but remains below the National Health Policy target of 2.5%**
- **Out-of-pocket expenditure on health, especially OPD, is a significant burden**
- **Scarcity of public hospitals, medical infrastructure and doctors forces many Indians to opt for private hospitals**

THE HEALTHCARE STORY of India is changing. More Indians today are covered by health insurance than ever before, and government hospitals are playing a bigger role in delivering affordable care. Yet, the cost of falling ill continues to climb.

The Survey on Household Social Consumption: Health, carried out by the National Sample Survey between January and December 2025, found that the landscape has significantly shifted since the previous survey in 2017-18, with more government initiatives to improve healthcare and coverage of health insurance. However, out-of-pocket expenditure, too, has risen substantially.

There was a substantial improvement in health insurance coverage across India between the two surveys—by 2025, 47% of rural and 44% of urban Indians,

respectively, had health insurance compared to a mere 14% in rural and 19% in urban, respectively, in 2017-18. Still, the average medical expenditure per hospitalisation rose from ₹20,135 in 2017-18 to ₹37,858 in 2025.

The results are not surprising given the strides in healthcare by both private players, non-government agencies and government initiatives. Schemes such as Pulse Polio, steps for reproductive, maternal and child health, sale of generic medicines at affordable prices through Jan Aushadhi stores, the Ayushman Bharat scheme, and now the focus on Ayushman Bharat Digital Mission to support integrated digital health infrastructure have improved health outcomes of 1.48 billion Indians.

Importantly, spending on health by both the Centre and states has increased over the years to about 1.8% of the GDP by FY26. This is evident in lower mortality rates and higher life expectancy rates.

Bhanu Prakash Kalmath S. J., Partner and Healthcare Industry Leader, Grant Thornton Bharat, says government health expenditure has increased from 28.6% of the total health expenditure in 2013-14 to 43.7% in 2022-23, while per-capita government spending has nearly tripled over the same period. “There has also been greater emphasis on primary care, financial protection, preventive health, early diagnosis, digital health infrastructure, and measures for social well-being,” he says.

But beyond this, there continues to be a dichotomy between the on-ground experience of patients and the intent of government schemes. India’s public healthcare is plagued by insufficient infrastructure. Experts highlight the need for even higher spending, noting that it remains below target.

Kalmath says given India’s large population and evolving healthcare needs, there is an opportunity to further increase healthcare expenditure to the National Health Policy target of 2.5% of GDP.

Sunil Sinha, Professor of Economics at Chandigarh-based Institute of Development and Communication, says as per the National Health Policy 2017, states are expected to spend 8% of their budget on health as it is a state subject. “But hardly any state is doing so,” he says. CAG’s recent Publication on State Finances 2024-25 reveals that the share of expenditure on health and family welfare by states was just ₹3.02 lakh crore, or 5.91% of their total expenditure of ₹51.2 lakh crore in FY25.

The National Health Accounts data for 2021-22 shows the erstwhile Jammu and Kashmir spent the highest on health—3% of its GSDP, while a developed state like Kerala was at 1.7% and Tamil Nadu at 1.3%, says Sinha. “There is no correlation between the overall economic performance of a state and its health spend-